

Pharyngitis & Bronchitis – Summary

Key Symptoms:

- **Acute Pharyngitis (AKA “Strep throat”)**: Low-grade fever, tonsillar exudate, painful throat, anterior cervical lymphadenopathy, absence of cough = suggestive of Group A step
- **Acute Bronchitis**: Cough up to 3-4 weeks +/- sputum production, wheezing, tachypnea, coryza, sore throat, malaise (self-limiting and resolves in 10-14 days, up to 3-4 weeks in some cases)

Most common causing-pathogens: Viral (>90% of cases) > bacterial (5-10% of cases)

- **Viral**: Rhinovirus, influenza A & B, parainfluenza, respiratory syncytial virus (RSV), adenovirus, coronavirus 1 to 3, enterovirus etc.
- **Bacterial**: *S. pneumoniae*, *H. influenzae*, *M. catarrhalis*, *M. pneumoniae*, *C. pneumoniae*, *B. pertussis*, *B. parapertussis* etc.
- See slide 14 to tell the difference between viral and bacterial pharyngitis (Modified Centor Score)

Non-Pharmacological Treatment: Education, ↑ humidity reduces cough, fluids, patient comfort

Pharmacological Treatment:

- **Symptom relief**: Analgesics, antipyretics, antitussives, bronchodilators, etc. (see slides 11-13)
- **Bronchitis**: No routine antibiotic therapy indicated for acute bronchitis, predominantly viral
- **Bacterial Pharyngitis**: Treatment directed towards GAHBS is recommended
 - **In children**
 - **1st line**: Penicillin V, amoxicillin x 10 days or penicillin G once intramuscularly
 - **Non-anaphylactic hypersensitivity reactions to penicillin**: amoxicillin challenge or cephalexin x 10 days
 - **Documented type 1 hypersensitivity to penicillin (anaphylaxis)**: clarithromycin x 10 days or azithromycin x 5 days or clindamycin x 10 days
 - **In adults**:
 - **First line**: Amoxicillin x 6-10 days or Penicillin V x 10 days or benzathine penicillin G IM x 1 dose (if risk of non-adherence)
 - **Patients with type 4 penicillin allergy or amoxicillin hypersensitivity (e.g. rash)**: cephalexin, clindamycin or clarithromycin x 10 days
 - **Documented type 1 hypersensitivity to penicillin (anaphylaxis)**: clarithromycin or azithromycin or clindamycin
 - **Refractory infections to first-line agents**: amoxicillin/clavulanic acid x 3 days or clindamycin x 3 days

Monitoring Parameters: Monitor for worsening symptoms

Additional Key Points:

- Majority of cases are of viral origin meaning antibiotics will not be indicated and treatment should be based on symptom management
- Purulent sputum indicates presence of leukocytes and inflammation NOT bacterial over viral infection
- Avoid antihistamines as their anticholinergic effects will prevent mucus secretions and its drainage